



Discharge from the Mental Health Inpatient Wards and Community Mental Health Teams Procedure

1. Scope

Site	Operational Area	Applicable to
Armadale Health Service	Mental Health Service	All disciplines

2. Purpose

Discharge planning is an integral part of a consumer's transition within mental health services, to the primary care setting or to other mental health or non-government organisation (NGO) sector services.

A Treatment, Support and Discharge Plan (TSDP) is developed in the inpatient and community setting in collaboration with the consumer, the personal support person (PSP), close family member and any other stakeholders, to ensure person-centred recovery, prevention or reduction of relapse and re-entry information. The discharge plan will incorporate an agreed Predicted Discharge Date (PDD).

Discharge planning should include consideration of the consumer's dependents and commitments including parents, children, and other caring roles.

Documentation of care provided to the consumer will use the relevant clinical documentation as per [State-wide Standardised Clinical Documentation \(SSCD\) for Mental Health Services](#).

3. Inpatient care services

3.1 Discharge

- Post discharge support is to be determined at the earliest point during admission. If a consumer is to be followed up by a community mental health team, this will be identified via the Multidisciplinary Team Review and/or stand-up meeting so that a Care Coordinator can be allocated. Referrals should be directed to the respective community team at the earliest opportunity to allow for care coordinator allocation and in reach.
- The allocated Care Coordinator will identify themselves to the consumer at the earliest opportunity prior to discharge wherever possible. Consideration of program involvement such as Crisis Resolution Home Treatment Team (CRHTT) or Transitional Care Unit (TCU) should be determined at the earliest point of admission.
- The PDD is documented in the consumer's Psychiatric Services Online Information System (WebPSOLIS) TSDP, and the patient administration system (webPAS).
- The treating team will regularly review the TSDP plans to ensure that the objectives of the admission have been met, where possible; and to modify the TSDP plans based on new and emerging information.

- Every consumer discharged will have a completed Discharge Summary entered into the Notifications and Clinical Summaries (NaCS) system on their day of discharge.
- Where only an interim Discharge Summary is available on the day of discharge, the final completed discharge summary will be sent to the GP as soon as practicable and ideally within 24 hours.
- Discharge information will be sent to the consumer's General Practitioner (GP) and to the service responsible for their ongoing care. Copies will be provided to the consumer and/or personal support person (PSP) unless:
 - The consumer's Psychiatrist reasonably believes that it is not in the consumer's best interests for them to receive a copy; or
 - The consumer does not consent to the inclusion of the PSP and the consumer's Psychiatrist considers that the refusal is reasonable.
- On discharge the consumer will receive a Care Transfer Summary/Discharge Summary, that will be completed on WebPSOLIS, which includes the following minimum dataset:
 - Contact details of emergency services
 - Out of hours contact numbers and other support services including emergency services numbers i.e., the Mental Health Emergency Response Line (MHERL)
 - Appointment card with details of the community mental health service appointment (if referred) and the name of the Care Coordinator
 - Will be informed they will receive a 7-day post discharge phone call from the Community service or the Care Coordinator
 - Information on the process for service re-entry to EMHS if needed
 - Consumer contact details – if unknown this must be stated on the Care Transfer Summary form.
- All consumers will have a Risk Assessment Management Plan (RAMP) and National Outcome and Case-mix Collection (NOCC) completed by the discharging clinician.
- Clinicians will be required to complete the:
 - Mental Health Discharge Checklist (PAKMR920)
 - Medical Mental Health Inpatient Discharge Checklist (TPAKMR1.9).
- The allocated clinician is responsible for entering the updated NOCC onto PSOLIS.
- Allocated Nursing clinician will contact community service via email on discharge with consumer's name, UMRN, brief isobar handover and confirmation that the RAMP and CTS is completed on WebPSOLIS with the medication list verified.
- All discharges should be scheduled to occur before 10:00hrs
- Discharges from CRHTT can occur at any time during operational hours as per CHRTT Doctor and consumer assessment outcome.

3.2 Unplanned Discharge

When a consumer seeks to discharge themselves there are three potential pathways:

- Consumer is safe to be discharged
- Consumer requires detention under the provisions of the MHA 2014; or
- the consumer should remain in hospital but chooses to Discharge Against Medical Advice.

- If the Duty Medical Officer/Registrar decides that a consumer may be discharged 'after hours' they must contact the on-call Consultant Psychiatrist and discuss the discharge before it occurs.
- If at the time of the request for discharge a medical practitioner or Psychiatrist is not available to review, then a risk assessment by an appropriately qualified mental health professional must be undertaken. If the risk is significant, this decision should be discussed with a senior mental health professional. A Senior Mental Health Professional may complete a [Form 2 – Order to Detain Voluntary Inpatient in Authorised Hospital for Assessment](#).
- If the consumer should remain in the hospital and is not detainable under the provisions of the MHA 2014 but chooses to leave the hospital, then the processes outlined in the [Discharge Against Medical Advice \(DAMA\) Policy](#) and associated [DAMA Guidelines](#) should be adhered to.
- A risk assessment and reason for discharge must be documented by the Duty Medical Officer/Registrar in the consumers healthcare record.
- The consumer's PSP must be involved in the decision to discharge before the PDD (unless the consumer does not consent to their involvement, or the treating psychiatrist believes that it is not in the consumer's best interests for them to be involved).
- On discharge the consumer and/or PSP will be provided with the following information:
 - Contact details of emergency services
 - 'Out of hours' contact numbers and other support services including emergency services numbers i.e. the Mental Health Emergency Response Line (MHERL)
 - Appointment card with details of the community mental health service appointment (if referred) and the name of the Care Coordinator; if not active with community teams, then referral can be made to Assessment and Treatment Team (ATT) based on risk for follow up
 - Appointment card with the details of the post discharge follow up appointment or Care Coordinator appointment
 - Information on the process for service re-entry to EMHS if needed.

3.3 Transfer of care for consumers on Clozapine

- If the consumer is on Clozapine and is being discharged to the Armadale Clinical Treatment Team (CTT) and Older Adult Community Team (OACT) the discharging nurse will contact the CTT Clozapine Coordinator or Care Coordinator to arrange the next Clozapine appointment, in line with the consumer's current Clozapine regime.
- Inpatient staff to ensure that the respective Community Mental Health service is provided with the Clozapine commencement date, level of monitoring required and copies of Echo/ECG reports.
- For transfer of clients who are not already registered with AHS Community teams the Inpatient team are to transfer the client via Clozapine Central or eCPMS to the respective clinic.
- The consumer must be discharged with the correct amount of Clozapine to last until their outpatient appointment. They will also be given a pathology form requesting a Full Blood Count, with instructions of when to have, prior to their next appointment.

4. Community mental health services

4.1 Seven-day follow-up

- The discharging inpatient nurse or CRHTT Clinician will advise the consumer they will receive a 7-day follow up contact post discharge, this includes consumers who reside outside of the catchment area.
- If the Community Care Coordinator plans to undertake post discharge follow-up within seven (7) days of discharge they are responsible for cancelling any other prearranged 7 day follow up contacts.
- Where the Care Coordinator is not available (e.g. on leave) post discharge follow up is the responsibility of the treating team.

4.2 Discharge or Transfer of Care from Community Mental Health Services

Successful and seamless transfers of care occur when there is collaborative planning and when information is shared with the relevant individuals/services involved. It is therefore expected that during treatment by the community mental health services, liaison with additional service providers occurs on a regular basis.

- On discharge from the service the treating Medical Officer is to provide written correspondence to GP and transfer of care to other mental health services. All discharge letters will have a header identifying the discharging team name at the top of the letterhead.
- Consideration should be given as to whether the consumer also receives a copy of the discharge letter.
- The Care Coordinator to provide written correspondence to the GP if the consumer has not been reviewed by a Medical Officer during their episode of care or has not engaged with the community service.
- The Care Coordinator is to complete a Care Transfer Summary if the consumer is transferring to another mental health service.
- All discharge letters to the GP will be uploaded on to PSOLIS by clerical staff at each clinic – refer to [Appendix 1](#).

Discharge letters from the Medical Officers are to include the following minimum dataset:

- Diagnosis
- Mental Health Act 2014 Status
- Management plan (including re-entry to service information)
- GP requests
- Ongoing management
- Physical health checks and monitoring
- Mental Health Care Plan
- Crisis plan and relapse signature
- Episode summary (including reason for referral)
- Diagnostic issues and formulation
- Risk assessment and management
- Management during the episode of care.

4.3 Documentation

The following documentation is required for all discharges/transfers from the community mental health teams:

- Care Transfer Summary (SMHMR916) (transfer to another service)
- Discharge PSOLIS Risk Assessment Management Plan (RAMP)
- Relevant NOCC measures
- PSOLIS Activation/Deactivation Form or Referral Outcome Form
- Discharge PSOLIS Management Plan/Crisis Plan – as appropriate
- Discharge Checklist
- Letter to GP (refer to the [Mental Health Information Hub](#) page for letter templates).

5. Legislative context

The following documents are required to give effect to this:

- *Mental Health Act 2014 (WA)*
- [Chief Psychiatrist's Standards for Clinical Care](#)
- [HDWA State-wide Standardised Clinical Documents for Mental Health Services OD0526/14](#)
- [HDWA Information Access, Use and Disclosure Policy MP0015/16](#)
- [EMHS Discharge Communication Policy](#)
- [EMHS Discharge Against Medical Advice Policy](#)
- [EMHS Care Coordination in Mental Health Policy](#)

6. Supporting information

The following documents support the implementation of this procedure:

- [HDWA Information Access, Use and Disclosure Policy Resource Compendium](#)
- [EMHS Discharge Against Medical Advice Guidelines](#)
- [Health Records Access and Management under the WA Mental Health Act 2014 Procedure](#)
- [Assessment and Treatment Team Process Manual](#)
- [Clinical Treatment Team Process Manual](#)

7. Compliance, monitoring and evaluation

Compliance with the requirements of this procedure will be monitored via the Mental Health Safety and Quality Committee. Compliance will be assessed via the following mechanisms:

- Healthcare record audit of discharge correspondence to the GP within seven (7) days of discharge – quarterly reporting
- Health Service Performance Report (HSPR) Indicator P1-1: Percentage of post discharge community care within 7 days following discharge from acute specialised mental health inpatient services – monthly (with a four-month lag).

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.10: The health service organisation supports clinicians to communicate with patients, carers, families and consumers about health and health care so that:
 - a. Information is provided in a way that meets the needs of patients, carers, families, and consumers
 - b. Information provided is easy to understand and use
 - c. The clinical needs of patients are addressed while they are in the health service organisation
 - d. Information needs for ongoing care are provided on discharge.
- Standard 6 – Action 6.3: Clinicians use organisational processes from the Partnering with Consumers Standard to effectively communicate with patients, carers, and families during high-risk situations to:
 - a. Actively involve patients in their own care
 - b. Meet the patient's information needs
 - c. Share decision-making.

9. Document owner

Service Director Mental Health

Enquiries relating to this procedure may be directed to:

Title: SRN7 Nurse Coordinator Mental Health
Department: Mental Health Service
Email: dene.olive@health.wa.gov.au

10. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
3	05/12/2023	05/12/2026	Scheduled review

11. Authorisation

This procedure has been authorised and issued by the Service Director Mental Health.

Authorised by	Karen Elliott – Service Director Mental Health
Authorisation date	04/12/2023
Published date	05/12/2023

Appendix 1: Uploading a discharge summary / letter in PSOLIS

A discharge summary is generated shortly after the time of the consumers discharge from an inpatient admission. These are generated from the Notification and Clinical Summaries (NaCS) system and will self-populate in the Psychiatric Services Online Information System (PSOLIS).

A discharge letter is generated after a client has been deactivated from a non-admitted service (e.g. Assessment Treatment Team (ATT) and Clinical Treatment Team (CTT)) and may be written by either the treating Medical Officer or the Care Coordinator. A discharge letter can be added to PSOLIS either by uploading a PDF (file must be named in PDF format using the prescribed naming conventions).

Document naming convention:

Use the following naming protocol when saving a PDF discharge letter into PSOLIS:

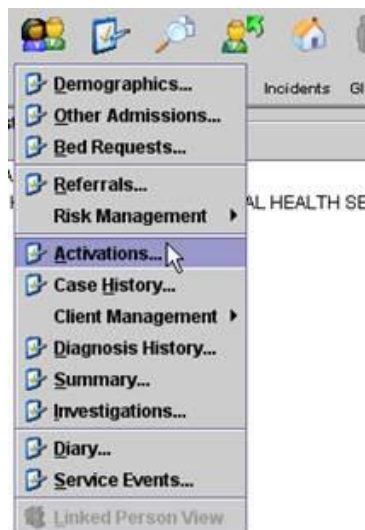
UMRN FAMILY NAME_Document Name_Date created [ddmmyy]

For example:

A1234567 SMITH_Discharge Letter_010120

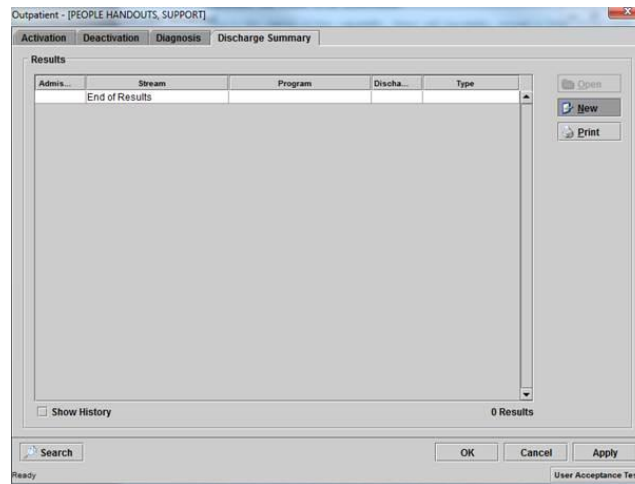
How to Upload Client's Discharge Letter

1. **Search** and **Open** the relevant client record
2. **Click on Person/Client** (twin heads) Icon and access **Activations** from the menu

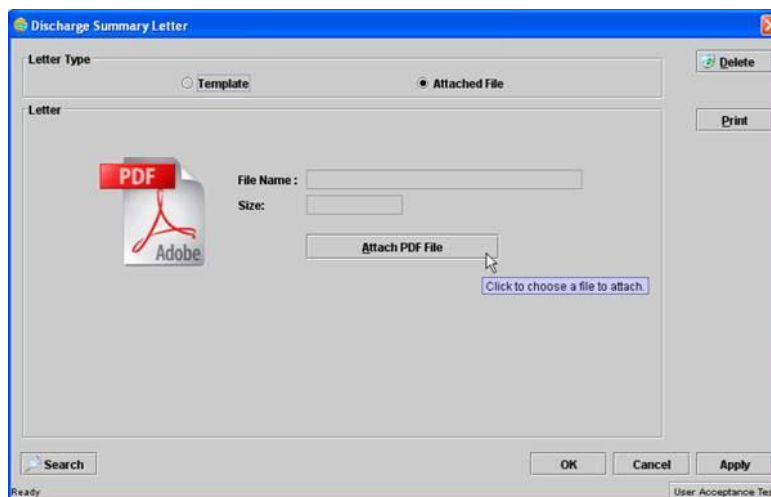
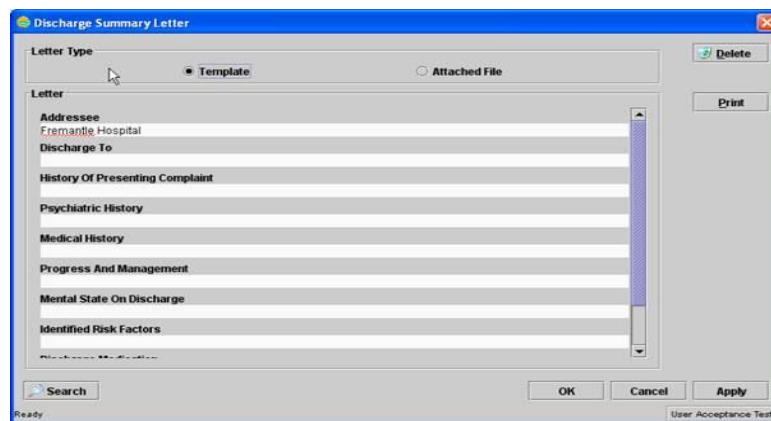


3. The List Client Activations page will appear. Select the relevant Activation and open relevant Activation (if the patient has already been deactivated or discharged you may need to use your 'ALL (Permitted Only)' button or the 'Show History' button).
4. From the **Activation Screen** select the Discharge Summary **Tab**
5. Click on **New** on right hand side of screen

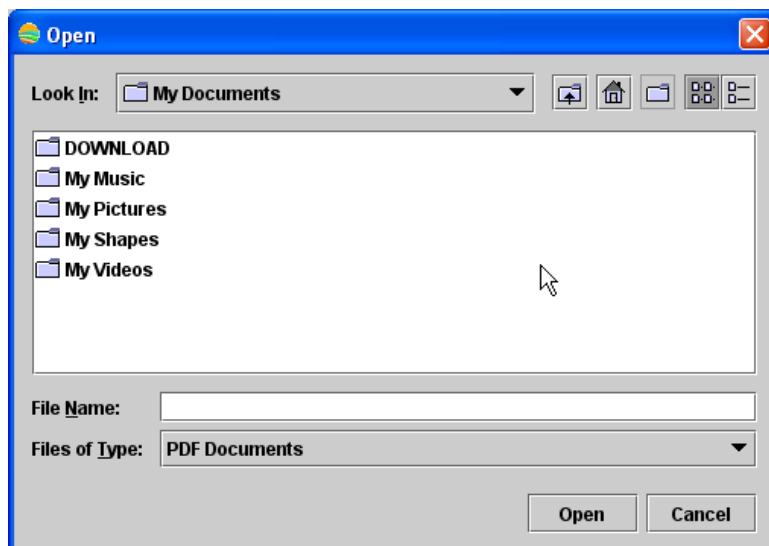
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6. The Discharge Summary Letter Screen will be deployed.
7. Select the **Attached File** to upload a PDF.



8. A search screen is deployed for searching available drives as per the image below:



9. Locate the required document to attach – check client details match, and naming convention is correct for saving the file.

Document naming convention:

Use the following naming protocol when saving a PDF discharge letter into PSOLIS:

UMRN FAMILY NAME_Document Name_Date created [ddmmyy]

For example:

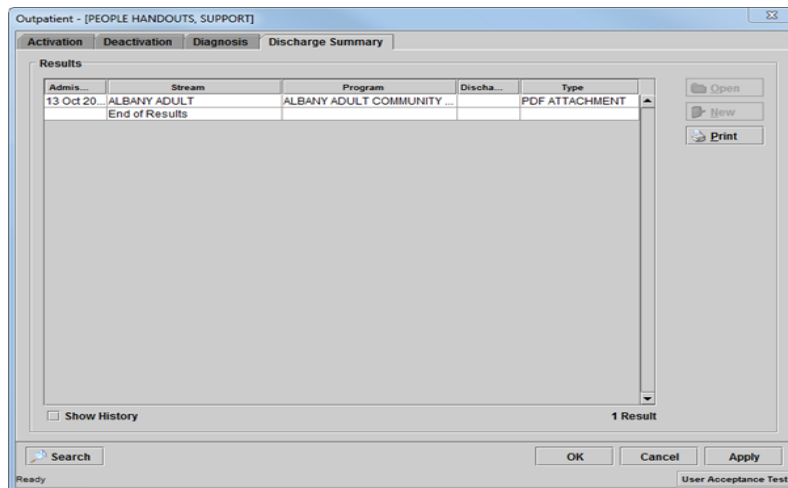
A1234567 SMITH_Discharge Letter_010120

10. Select file and click on **Open**.

User is returned to the **Discharge Summary Letter Screen** showing the PDF file name and size of the attached document.



11. Click on **OK** – The attached PDF will now show in the results table.



12. Click on **Apply** to Save, or **OK** to save and close the screen.

The User is returned to the List Client Activations Screen.

To view or print the attached PDF

1. Access the **Activations** via the process described in the previous pages.
2. On the **Discharge Summary Page** select the required document from the table shown.
3. Click on the **Print** button. This will open the letter for either viewing or printing as required.

Discharge Summaries or Discharge Letters may also be accessed via the Client Management Menu (select Discharge Summary) – separate instructions for this are available from the PSOLIS Coordinator.